

SENATE No. 789

The Commonwealth of Massachusetts

PRESENTED BY:

Liz Miranda

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act relative to insurance coverage for doula services.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	
<i>Liz Miranda</i>	<i>Second Suffolk</i>	
<i>Bruce E. Tarr</i>	<i>First Essex and Middlesex</i>	<i>4/2/2025</i>
<i>John F. Keenan</i>	<i>Norfolk and Plymouth</i>	<i>4/22/2025</i>

SENATE No. 789

By Ms. Miranda, a petition (accompanied by bill, Senate, No. 789) of Liz Miranda for legislation relative to insurance coverage for doula services. Financial Services.

[SIMILAR MATTER FILED IN PREVIOUS SESSION
SEE SENATE, NO. 680 OF 2023-2024.]

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act relative to insurance coverage for doula services.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 32A of the General Laws is hereby amended by inserting after
2 section 33 the following section:- Section 34. (a) For the purpose of this section, the term “doula
3 services” shall have the following meaning:

4 “Doula Services” are physical, emotional, and informational support, but not medical
5 care, provided by trained doulas to individuals and families from conception until twelve months
6 after pregnancy, labor, childbirth, adoption, miscarriage, stillbirth or abortion. Doula services
7 include but are not limited to: (1) continuous labor and delivery support, inclusive of all
8 outcomes; (2) support for pregnancy loss or infant loss, including bereavement home or in-
9 person visits; (3) accompanying individuals to health care and social services appointments; (4)
10 connecting individuals to community-based and state- and federally-funded resources, including

those which address social determinants of health; (5) being on-call around the time of birth, adoption, loss or abortion, as well as providing on-call support in-person or via telehealth for individuals' questions or concerns; (6) support for other individuals providing care for a birthing or adoptive parent, including spouses, partners, and other family members. (7) educational and informational support including but not limited to childbirth preparation, newborn care, perinatal mental health, and postpartum recovery. (8) empowering individuals to advocate for their preferences, needs and rights during pregnancy, labor and delivery, and postpartum.

(b) Any coverage offered by the commission to an active or retired employee of the commonwealth and their dependents insured under the group insurance commission (hereinafter "policy") shall provide coverage for all doula services. Coverage provided for doula services shall not be subject to any deductible, coinsurance, copayment or any other cost-sharing requirement. (c) No policy shall require a referral for doula services by any other health care provider as a condition of reimbursement. (d) Doulas shall be reimbursed for their services at a rate at least equivalent to MassHealth's reimbursement rate for doula services. (e) A policy shall cover, without prior authorization, a minimum of twenty hours of prenatal and postpartum doula services per pregnancy, and continuous support throughout labor and delivery, inclusive of all pregnancy outcomes. Within the authorized number of doula hours, policies shall not impose limitations on the amount of time a doula can bill per patient per day. (f) Policies must establish a process to approve coverage of additional hours of doula services in cases where the patient has heightened risk or need. (g) Policies shall follow the doula credentialing requirements developed by the Massachusetts Department of Public Health, and may not impose any additional credentialing requirements. Additionally, any doula approved as a MassHealth Doula Provider shall be deemed as meeting all credentialing requirements.

SECTION 2. Chapter 175 of the General Laws, as so appearing, is hereby amended by inserting after section 47UU the following section:- SECTION 47VV. (a) For the purpose of this section, the term “doula services” shall have the following meaning: “Doula Services” are physical, emotional, and informational support, but not medical care, provided by trained doulas to individuals and families from conception until twelve months after pregnancy, labor, childbirth, adoption, miscarriage, stillbirth or abortion. Doula services include but are not limited to: (1) continuous labor and delivery support, inclusive of all outcomes; (2) support for pregnancy loss or infant loss, including bereavement home or in-person visits; (3) accompanying individuals to health care and social services appointments; (4) connecting individuals to community-based and state- and federally-funded resources, including those which address social determinants of health; (5) being on-call around the time of birth, adoption, loss or abortion, as well as providing on-call support in-person or via telehealth for individuals’ questions or concerns; (6) support for other individuals providing care for a birthing or adoptive parent, including spouses, partners, and other family members.

(7) educational and informational support including but not limited to childbirth preparation, newborn care, perinatal mental health, and postpartum recovery. (8) empowering individuals to advocate for their preferences, needs and rights during pregnancy, labor and delivery, and postpartum. (b) A policy, contract, agreement, plan or certificate of insurance issued, delivered or renewed within the commonwealth (hereinafter “policy”) shall provide coverage for all doula services. Coverage provided under this section for doula services shall not be subject to any deductible, coinsurance, copayment or any other cost-sharing requirement. (c) No policy shall require a referral for doula services by any other health care provider as a condition of reimbursement. (d) Doulas shall be reimbursed for their services at a rate at least

equivalent to MassHealth's reimbursement rate for doula services. (e) A policy shall cover, without prior authorization, a minimum of twenty hours of prenatal and postpartum doula services per pregnancy, and continuous support throughout labor and delivery, inclusive of all pregnancy outcomes. Within the authorized number of doula hours, policies shall not impose limitations on the amount of time a doula can bill per patient per day. (f) Policies must establish a process to approve coverage of additional hours of doula services in cases where the patient has heightened risk or need.

(g) Policies shall follow the doula credentialing requirements developed by the Massachusetts Department of Public Health, and may not impose any additional credentialing requirements. Additionally, any doula approved as a MassHealth Doula Provider shall be deemed as meeting all credentialing requirements.

SECTION 3. Chapter 176A of the General Laws, is hereby amended by inserting after section 8VV the following section:- SECTION 8WW. (a) For the purpose of this section, the term "doula services" shall have the following meaning: "Doula Services" are physical, emotional, and informational support, but not medical care, provided by trained doulas to individuals and families from conception until twelve months after pregnancy, labor, childbirth, adoption, miscarriage, stillbirth or abortion. Doula services include but are not limited to: (1) continuous labor and delivery support, inclusive of all outcomes; (2) support for pregnancy loss or infant loss, including bereavement home or in-person visits; (3) accompanying individuals to health care and social services appointments; (4) connecting individuals to community-based and state- and federally-funded resources, including those which address social determinants of health;

(5) being on-call around the time of birth, adoption, loss or abortion, as well as providing on-call support in-person or via telehealth for individuals' questions or concerns; (6) support for other individuals providing care for a birthing or adoptive parent, including spouses, partners, and other family members. (7) educational and informational support including but not limited to childbirth preparation, newborn care, perinatal mental health, and postpartum recovery. (8) empowering individuals to advocate for their preferences, needs and rights during pregnancy, labor and delivery, and postpartum. (b) Any contract between a subscriber and a corporation subject to this chapter, pursuant to an individual or group hospital service plan that is delivered, issued or renewed within the commonwealth (hereinafter "policy") shall provide coverage for all doula services. Coverage provided under this section for doula services shall not be subject to any deductible, coinsurance, copayment or any other cost-sharing requirement. (c) No policy shall require a referral for doula services by any other health care provider as a condition of reimbursement. (d) Doulas shall be reimbursed for their services at a rate at least equivalent to MassHealth's reimbursement rate for doula services.

(e) A policy shall cover, without prior authorization, a minimum of twenty hours of prenatal and postpartum doula services per pregnancy, and continuous support throughout labor and delivery, inclusive of all pregnancy outcomes. Within the authorized number of doula hours, policies shall not impose limitations on the amount of time a doula can bill per patient per day. (f) Policies must establish a process to approve coverage of additional hours of doula services in cases where the patient has heightened risk or need. (g) Policies shall follow the doula credentialing requirements developed by the Massachusetts Department of Public Health, and may not impose any additional credentialing requirements. Additionally, any doula approved as a MassHealth Doula Provider shall be deemed as meeting all credentialing requirements.

SECTION 4. Chapter 176B of the General Laws, is hereby amended by inserting after section 4VV the following section:- Section 4WW. (a) For the purpose of this section, the term “doula services” shall have the following meaning: “Doula Services” are physical, emotional, and informational support, but not medical care, provided by trained doulas to individuals and families from conception until twelve months after pregnancy, labor, childbirth, adoption, miscarriage, stillbirth or abortion. Doula services include but are not limited to: (1) continuous labor and delivery support, inclusive of all outcomes;

(2) support for pregnancy loss or infant loss, including bereavement home or in-person visits; (3) accompanying individuals to health care and social services appointments; (4) connecting individuals to community-based and state- and federally-funded resources, including those which address social determinants of health; (5) being on-call around the time of birth, adoption, loss or abortion, as well as providing on-call support in-person or via telehealth for individuals’ questions or concerns; (6) support for other individuals providing care for a birthing or adoptive parent, including spouses, partners, and other family members. (7) educational and informational support including but not limited to childbirth preparation, newborn care, perinatal mental health, and postpartum recovery. (8) empowering individuals to advocate for their preferences, needs and rights during pregnancy, labor and delivery, and postpartum. (b) Any subscription certificate under an individual or group medical service agreement that is delivered, issued or renewed within the commonwealth (hereinafter “policy”) shall provide coverage for all doula services. Coverage provided under this section for doula services shall not be subject to any deductible, coinsurance, copayment or any other cost-sharing requirement.

(c) No policy shall require a referral for doula services by any other health care provider as a condition of reimbursement. (d) Doulas shall be reimbursed for their services at a rate at

least equivalent to MassHealth’s reimbursement rate for doula services. (e) A policy shall cover, without prior authorization, a minimum of twenty hours of prenatal and postpartum doula services per pregnancy, and continuous support throughout labor and delivery, inclusive of all pregnancy outcomes. Within the authorized number of doula hours, policies shall not impose limitations on the amount of time a doula can bill per patient per day. (f) Policies must establish a process to approve coverage of additional hours of doula services in cases where the patient has heightened risk or need. (g) Policies shall follow the doula credentialing requirements developed by the Massachusetts Department of Public Health, and may not impose any additional credentialing requirements. Additionally, any doula approved as a MassHealth Doula Provider shall be deemed as meeting all credentialing requirements.

SECTION 5. Chapter 176G of the General Laws, is hereby amended by inserting after section 4NN the following section:- Section 4OO. (a) For the purpose of this section, the term “doula services” shall have the following meaning:

“Doula Services” are physical, emotional, and informational support, but not medical care, provided by trained doulas to individuals and families from conception until twelve months after pregnancy, labor, childbirth, adoption, miscarriage, stillbirth or abortion. Doula services include but are not limited to: (1) continuous labor and delivery support, inclusive of all outcomes; (2) support for pregnancy loss or infant loss, including bereavement home or in-person visits; (3) accompanying individuals to health care and social services appointments; (4) connecting individuals to community-based and state- and federally-funded resources, including those which address social determinants of health; (5) being on-call around the time of birth, adoption, loss or abortion, as well as providing on-call support in-person or via telehealth for individuals’ questions or concerns; (6) support for other individuals providing care for a birthing

or adoptive parent, including spouses, partners, and other family members. (7) educational and informational support including but not limited to childbirth preparation, newborn care, perinatal mental health, and postpartum recovery. (8) empowering individuals to advocate for their preferences, needs and rights during pregnancy, labor and delivery, and postpartum.

(b) Any individual or group health maintenance contract that is issued or renewed within or without the commonwealth (hereinafter “policy”) shall provide coverage for all doula services. Coverage provided under this section for doula services shall not be subject to any deductible, coinsurance, copayment or any other cost-sharing requirement. (c) No policy shall require a referral for doula services by any other health care provider as a condition of reimbursement. (d) Doulas shall be reimbursed for their services at a rate at least equivalent to MassHealth’s reimbursement rate for doula services. (e) A policy shall cover, without prior authorization, a minimum of twenty hours of prenatal and postpartum doula services per pregnancy, and continuous support throughout labor and delivery, inclusive of all pregnancy outcomes. Within the authorized number of doula hours, policies shall not impose limitations on the amount of time a doula can bill per patient per day. (f) Policies must establish a process to approve coverage of additional hours of doula services in cases where the patient has heightened risk or need. (g) Policies shall follow the doula credentialing requirements developed by the Massachusetts Department of Public Health, and may not impose any additional credentialing requirements. Additionally, any doula approved as a MassHealth Doula Provider shall be deemed as meeting all credentialing requirements.

SECTION 6. Sections 2 through 6 of this act shall apply to all policies, contracts and certificates of health insurance subject to chapters 32A, chapter 175, chapter 176A, chapter 176B, and chapter 176G which are delivered, issued or renewed on or after September 1, 2026.

SECTION 7. Doula Advisory Committee: There is hereby created a Doula Advisory Committee.

(1) The committee shall consist of 10-12 members to be appointed by the Governor, or designee.

(a) All but 2 of the members shall be practicing doulas from the community representing a range of experience levels and a diversity of lived experience; the remaining 2 members shall be individuals from the community who have experienced pregnancy as a MassHealth member and are not practicing doulas. (b) The members of the committee shall represent an equitable

geographic distribution from across the Commonwealth, including representation from areas within the Commonwealth where maternal and infant outcomes are worse than the state average, as evidenced by the MA Department of Public Health's most current perinatal data available at the time the member is appointed. (2) The committee shall be convened within six months of passage of this law. (3) Of the initial appointments to the Doula Advisory Committee, half shall be appointed to a term of 2 years and half shall be appointed to a term of 18 months. Thereafter, all terms shall be 2 years. The Governor, or designee, shall fill vacancies as soon as practicable.

(4) At least once every 8 weeks, the Division of Medical Assistance shall meet with the Doula Advisory Committee to consult about MassHealth's coverage of doula services, including but not limited to the following: (a) the standards and processes around billing for and prompt reimbursement of doula services; (b) establishing grievance procedures for doulas, MassHealth members, and health care providers about MassHealth's coverage of doula services, the provision of doula services to MassHealth members, and bias that doulas face as they try to integrate into birth teams; (c) maintaining a reimbursement rate for doula services that incentivizes and supports a diverse workforce representative of the communities served, and establishing a recurring timeframe to review that rate in light of inflation and changing costs of living in the commonwealth; (5) Each year, the Doula Advisory Committee must, by a majority

194 vote of a quorum of its members, select an individual to serve as its chairperson for a one year
195 term. The Doula Advisory Committee may replace the chairperson in the same manner mid-
196 term. (6) The Doula Advisory Committee may, by a majority vote of a quorum of its members,
197 reduce the frequency of meetings with MassHealth to less than once every 8 weeks. (7) The
198 Division of Medical Assistance shall seek resources to offer reasonable compensation to
199 members of the Doula Advisory Committee for fulfilling their duties, and shall reimburse
200 members for actual and necessary expenses incurred while fulfilling their duties.

201 SECTION 8. Chapter 111 of the General Laws is hereby amended by inserting in section
202 70E after “Every patient or resident of a facility shall have the right.”: (i) to have their birth
203 doula’s continuous presence during labor and delivery. Facilities shall not place an undue burden
204 on access of a patient’s doula to clinical labor and delivery settings, and shall not arbitrarily
205 exclude a patient’s doula from such settings. A doula shall not be counted as a patient's guest or
206 support person.